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Request for Information: *Health Technology Ecosystem*

About CLEAR

CLEAR is a secure identity company transforming how individuals verify their identity across digital and physical experiences. Our connected identity platform enables more than 31 million Members to use their face to instantly authenticate their identity—powering secure, seamless access in high-stakes environments.

Well known for our presence in 59 U.S. airports through CLEAR+, and as a TSA PreCheck® enrollment provider, CLEAR also delivers high-assurance digital identity solutions across healthcare, financial services, and retail. We partner with leading organizations including The Home Depot, LinkedIn, T-Mobile, and Uber, while improving the efficiency and security of health systems such as University of Miami Health, Wellstar Health, and Community Health Network. Additionally, we have integrations underway with leading software providers including Docusign and Greenhouse.

CLEAR in Healthcare

With more than 15 years of experience simplifying and securing access, CLEAR offers a trustworthy path forward for the healthcare industry. Trusted in zero-fail environments, CLEAR is designed to make identity verification simple, secure, and scalable.

CLEAR brings enterprise-grade, consumer-centric identity experiences to healthcare to improve access, reduce administrative burden, and strengthen security. We are trusted by national healthcare platforms like Epic (with CLEAR "Under Construction" in Epic Toolbox and powering live Vendor Services integrations), Surescripts, and InterSystems.

Health systems, such as Community Health Network, Wellstar Health, and University of Miami Health, use CLEAR to replace outdated verification methods. CLEAR reduces operational costs and prevents fraud by replacing vulnerable, knowledge-based authentication and manual verification with high-fidelity biometric verification. At Community Health Network, CLEAR drove a 1.5x higher verification success rate (93% vs. 62%) and reduced account-related support calls by more than 54%.

CLEAR’s network is rapidly expanding—connecting payers, providers, and digital health innovators with populations across the country.

Standards-Based, High-Assurance Identity

CLEAR is approved by the Kantara Initiative as meeting NIST Identity Assurance Level 2 (IAL2) and Authenticator Assurance Level 2 (AAL2) standards. IAL2 certification ensures identity verification includes document-based proofing and anti-spoofing protections, offering robust defense against impersonation and fraud.

Breaches are becoming more frequent and more expensive. Eighty percent of breaches start with compromised credentials — and while the average cost of a breach in the US is $4.88 million, the average cost of a healthcare breach is twice that at $9.77 million. Standard, device-based multi-factor authentication (MFA) is no longer sufficient to combat the evolving threats to our nation's health networks. More can, and should, be done to protect patient data. Bad actors exploit the status quo by leveraging sophisticated social engineering tactics through call centers to spoof individuals and gain access to their credentials and personal data. Many healthcare organizations have faced both near misses and breaches, compromising the credentials of patients and providers. There are documented cases of criminals successfully circumventing MFA through advanced attacks, defeating individuals' last line of defense.

Health systems and other HIPAA Covered Entities have already begun to determine that legacy MFA is no longer strong enough for use cases such as account creation and password reset. Phishing attacks, SIM Swapping and Man-in-the-Middle (MitM) attacks are just some of the ways legacy MFA systems are defeated in successful data breaches and ransomware attacks. IBM's *2024 Cost of a Data Breach* report found that phishing and stolen or compromised credentials are the two most prevalent attack vectors. The Cybersecurity and Infrastructure Security Agency (CISA) provides guidance for implementing phishing-resistant MFA, calling it "the most secure form of MFA".

CISA identifies biometrics as one of the factors utilized in phishing-resistant MFAs. According to Dr. David Reis, CIO at University of Miami Health, "Multi-factor Authentication verifies devices, not people. This is a known problem which MFA wasn't built to address. It's technology like CLEAR's that gives us the automated ability to verify the human which closes a big security gap."

The Office of the National Coordinator (ONC) has a history of leveraging federal standards for identity verification, including IAL2 as a standard for Individual Access Services through the Trusted Exchange Framework and Common Agreement (TEFCA). Establishing an IAL2 standard for providers benefits everyone, as CLEAR and other Kantara-certified IAL2 organizations have already identity-proofed many consumers at the IAL2 level.

These certifications support secure, consumer-mediated data exchange and unlock better experiences, such as:

- Single, reusable identity across payers, providers, and health tools
- Reliable authentication for account access and recovery
- Reduced reliance on passwords and portal-specific logins

CLEAR also maintains SOC 2 Type 2 and PAD-2 certifications, ensuring compliance with industry-leading security and privacy standards.

In practice, IAL2 enables scalable, interoperable identity frameworks—so that, as an example, a Medicare beneficiary could access multiple services (from account recovery to Blue Button to digital health tools) using the same, verified identity. This reduces the need for multiple logins, unlocks better experiences and reduces fraud across the ecosystem.

Value of a Reusable, Networked Digital Identity

A reusable, networked digital identity enhances patient privacy as well as processing efficiency by removing friction for returning users and tying identity to unique user biometrics. This ensures that each person is uniquely proofed and identified across the network based on who they are rather than what they know (i.e., username and password). As a user, once you have enrolled at any point in the system, you never need to go through that friction again. Simultaneously, healthcare organizations eliminate the risk of duplicate records and share the burden of onboarding users across the network so that there is no need to re-enroll a user every time they move from one service to another, reducing overall cost, user friction, and increasing processing efficiency.

Core benefits of a networked digital identity for healthcare organizations include:

1. **Convenience and Speed**: A reusable networked identity leverages biometric data to streamline identity verification. Users can prove who they are once at high fidelity (IAL2) and carry that identity across the network which improves efficiency and reduces fraud.
2. **Reusable Identity**: A reusable networked identity allows users to create a single, verified digital identity that can be used across various platforms and services. This eliminates redundant identity checks and manual data entry, improving user experience and reducing the risk of drop-offs in onerous processes.
3. **Cost Savings:** A reusable identity means that the cost of identity proofing is amortized across the network of providers. Providers that adopt a reusable identity can leverage verified experiences that cost a fraction of legacy identity verification platforms.
4. **Improve Patient Matching & Reduce Duplicates:** Demographic information (i.e. name, DOB, address) are verified and corroborated with 3rd party sources, leading to a high patient match rate and reducing duplicate health records.
5. **Identity Orchestration:** Different experiences should command different levels of assurance. For example, a lightweight verification may be appropriate for patient scheduling, whereas a document-based level of assurance is appropriate for account creation or check-in, with IAL2 being reserved for the highest risk use cases such as password reset, individual access, and Electronic Prescribing of Controlled Substances. Lightweight verification, for example, would not be sufficient for accessing personal health information (PHI), so it is important that a rule sets appropriate standards depending on the sensitivity and security importance of the actions.

6. **Security and Privacy**: The importance of data security and privacy, whether at rest or in transit, is paramount given the sensitive nature of healthcare data. Users need to understand and agree to what data they are sharing. CLEAR, for example, emphasizes strong privacy protections and secure data handling. Our platform allows users to control what personal information is shared, while maintaining high security standards within the industry, and with NIST and DHS. We never share data without explicit user consent. This protects sensitive information and mitigates risks like identity theft. CLEAR maintains an audit trail of every third-party that a CLEAR Member has consented to share their data with, and are in control of purging data connections if desired.

Commitment to Security, Interoperability, Governance and Consumer Choice

Security is our top priority and has been foundational to CLEAR since day one — and safety and transparency are at the center of everything we do. Our comprehensive security program meets the highest standards for data protection and privacy, and our secure identity platform is built to protect personal information and keep users in control. Learn more here.

As a proud member of the CARIN Alliance, CLEAR is actively shaping governance, trust, and interoperability frameworks for patient-directed exchange. We support neutral, standards-aligned identity infrastructure such as NIST IAL2, ensuring that patients and providers can access and share health data securely without being locked into vendor-specific solutions.

CLEAR supports the establishment of NIST IAL2 as a federal standard, including for healthcare, and believes that consumers should have the freedom to choose their preferred high-assurance, interoperable identity brand that puts them in control of their data.

CLEAR's Response to RFI Questions

PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)):

a. **What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?**
    i. **Problem:** Patients may face challenges in accessing digital identity credentials due to lack of awareness, limited digital literacy, or discomfort with new technologies. Additionally, inconsistent implementation across healthcare touchpoints may create friction and reduce perceived value.
        1. **Solution:** CLEAR is a trusted consumer brand with more than 31 million existing, verified users. Existing users recognize the CLEAR brand and can verify instantly, which accelerates adoption and conversion.
        2. **Solution:** CLEAR helps overcome these challenges by making it easy for users to enroll and re-authenticate using a simple, mobile-first interface. CLEAR's biometric identity platform is more reliable than knowledge-based authentication, with higher conversion rates and greater

assurance, as demonstrated in the Community Health Network case study.

3. **Solution:** CLEAR creates a frictionless identity experience across digital and physical channels, reducing friction and boosting engagement with portals and virtual care platforms.
4. **Solution:** CLEAR supports CMS's focus on providing patients with optionality, which could include in-person verification, such as CLEAR at airports or Staples stores.

ii. **Problem:** Healthcare organizations either leverage legacy systems (that rely on Knowledge-Based Authentication or MFA) or have not made modern, federated digital identity platforms readily available to consumers (i.e. patient portal). This is due to multiple factors including health IT vendors not offering this as part of their core service, capital budget challenges to implement the technology, and lack of government incentives to push this effort.

1. **Solution:** CLEAR supports CMS's commitment to implement a modern digital identity. This action encourages organizations in the private sector (e.g. health systems, payers) to adopt similar technologies, which would improve patient pass rates and thereby adoption of digital tools. Additionally, CMS's actions would reduce the cost for other healthcare organizations, given network effects of federated identity platforms. CLEAR would welcome the opportunity to design a business model that minimizes cost burdens after an identity has been verified within the healthcare ecosystem, especially if consumers, once verified, can "write" their unique ID within a certified Electronic Health Record (EHR).

b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

i. Widespread use of digital identity credentials can streamline and improve patient access across many use cases. Currently, healthcare organizations leverage several point solutions to triangulate and verify patient identity, including manual patient lookup, call centers, and knowledge-based questions. This current state leads to high cost and operational inefficiency for both patients and provider organizations. Additionally, this fragmentation prevents patients from easily accessing and sharing their own records. Modern, federated digital identity technologies like CLEAR allow patients to enroll once with a healthcare organization, and reuse their identity across many use cases. This improves the experience and drives operational efficiency for healthcare organizations. Here are some examples of use cases and the benefits realized:

1. **Account Creation:** online portals require patients to register with several identifiers (eg self-attested data, knowledge-based authentication questions) that add significant friction and are insecure. Modern identity platforms like CLEAR streamline the experience and reduce call center volume. For example, Community Health Network has seen a 1.5x increase in conversion and 54% reduction in call center calls with CLEAR (case study).

2. **Account Recovery:** When a patient forgets their password, they are guided through long, complex recovery processes that can vary from provider to provider. With multifactor authentication (MFA) and Knowledge-Based Authentication (KBA) as primary verification methods, these practices are vulnerable to fraudsters and social engineering attacks, which has become a significant challenge across healthcare. With technologies like CLEAR, patients can take a simple selfie to reset their password.
3. **Check-In:** Whether patients use digital or paper registration forms, the check-in process can be long and repetitive. As systems become more fragmented with time, patients may be asked to sign the same forms more than once in a single appointment. With technologies like CLEAR, patients can check-in with just a selfie on their smartphone or at a clinic (i.e. at a kiosk).
4. **Access Health Records:** When patients must rely on call centers to access their records or receive support, verifying their identity can be time consuming — and easily compromised. Plus it is operationally inefficient and expensive for health systems to maintain. With technologies like CLEAR, a user can choose their preferred IAL2-certified provider to verify their identity in order to safely and securely access and share their comprehensive health record with their provider, caregiver, etc.

ii. CLEAR's privacy-preserving technology is built to protect information and keep users in control. CLEAR's Privacy Commitment is available here.

iii. Reusable identity platforms like CLEAR create compounding network effect benefits across the healthcare ecosystem—patients do not need to verify multiple times, and identity assurance scales across use cases. As more entities use the same high-assurance credential, the network effect grows, connecting CMS, providers, and payers to high-value populations nationwide.

c. What are the potential downsides?

i. **Problem:** Underserved populations may be disproportionately impacted if identity solutions are not inclusive, multilingual, and mobile-friendly. There may also be concerns about privacy or surveillance if identity credentials are not transparently governed and patient-controlled.

1. **Solution:** CLEAR is designed to be equitable for all users, offering multilingual support, mobile-first tools with accessibility features, and pathways to human assistance. It meets users where they are to ensure inclusive identity verification for diverse Medicare beneficiaries. For example, Healthix launched Individual Access Services to empower consumers with free, easy and secure access to their healthcare data from more than 8,000 health facilities across New York State in a single portal.
2. **Solution:** CLEAR drove 97% adoption among frontline healthcare workers at Community Health Network, showing success across user types.

ii. **Problem:** Certain populations of people may not be inclined to use digital-first solutions, which could include: elderly users or those who do not have access to smartphones.
    1. **Solution:** CLEAR works with several health systems who have seen that ~40% of consumers who use CLEAR are at least 65-years old.
    2. **Solution:** CLEAR supports CMS's focus on providing patients with optionality, which could include in-person verification, such as CLEAR at airports and Staples stores.

d. How would encouraging the use of CSPs improve access to health information?

i. Credential service providers (CSPs) following NIST IAL2/AAL2 provide an industry consensus standardized approach already recognized by the federal government when consumers interact with secure websites such as IRS, VA and SSA. As a federal standard that associates both verified identity and patient demographics, this approach promotes market competition and ensures much greater patient demographic data quality when consumers register new accounts, request password recovery, or consent to data sharing, among other use cases.

ii. Expansion of the NIST IAL2/AAL2 standard to healthcare, such as CMS's commitment to bringing modern identity verification processes to Medicare.gov, would extend the value of the reusable networked identity credential to improving access to healthcare information.

iii. Modern identity verification solutions provide many benefits to access including:
    1. **Convenience and Speed**: A reusable networked identity leverages biometrics and verified demographics to streamline identity verification for key access use cases including account creation and password reset. Users can prove who they are once at high fidelity (IAL2) and carry that identity across the network which improves efficiency and reduces fraud. It also reduces healthcare's reliance on manual verification and operational costs (i.e. call centers).
    2. **Reusable Identity**: A reusable networked identity allows users to create a single, verified digital identity that can be leveraged across various platforms and services. This eliminates redundant identity checks and manual data entry, improving user experience and reducing dropoff.
    3. **Improve Patient Matching & Reduce Duplicates:** Demographic information (i.e. name, DOB, address) are verified and corroborated with 3rd party sources, leading to a high patient match rate and reducing duplicate health records.
    4. **Security and Privacy**: Healthcare data security is critical, and has never been more important given the cybersecurity issues facing healthcare. Users need to understand and agree to what data they are sharing. CLEAR emphasizes strong privacy protections and secure data handling. Our platform allows users to control what personal information is shared, while maintaining high security standards within the industry, and with NIST and DHS. We never share data without explicit user consent. This protects sensitive information and mitigates risks like identity theft.

CLEAR maintains an audit trail of every third-party that a CLEAR Member has consented to share their data with, and are in control of purging data connections if desired.

e. **What role should CMS/payers, providers, and app developers have in driving adoption?**
   i. **CMS:** CLEAR supports CMS's commitment to implementing a federated, modern identity verification solution for Medicare. This critical infrastructure investment means that Medicare beneficiaries can enroll through Medicare.gov and reuse their digital identity credential at their health system, helping improve the patient experience at their local health system, as well as helping reduce private sector costs in re-verifying given network effects of modern identity verification solutions. This not only sends an important signal to the healthcare market on the importance of digital identity, but will also help reduce costs for other organizations (i.e. health systems) to implement this technology given network effects of reusable identity platforms like CLEAR.
   ii. **Payers, Providers, App Developers:** These organizations should prioritize federated, modern identity verification solutions for digital use cases (e.g. account creation, account recovery), ensuring that consumers are verified when accessing PHI, while also setting the foundation for reusability for additional experiences (eg check-in, prescription pickup).

f. **How can CMS encourage patients to get digital identity credentials?**
   i. CMS can demonstrate the reduced consumer friction when using digital identity credentials to access MyMedicare.gov, among other CMS digital services, through the adoption of federated, modern identity verification solutions for digital use cases (e.g. account creation, account recovery) for federal programs. This has a multi-pronged value proposition of a) incenting Medicare beneficiaries to leverage modern identity verification solutions that they can reuse elsewhere such as their local health system; b) reducing fraud and improving operational efficiency (e.g. eliminating unnecessary call center calls); and c) serving as a signal to industry that digital identity is a valuable and needed tool.
   ii. CMS could require NIST IAL2 identity verification for certain payer and provider use cases who treat Medicare patients (e.g. account creation, password reset, pickup of a controlled substance at the pharmacy). CLEAR champions neutral, standards-aligned identity infrastructure to ensure that patients can access and share health data securely and seamlessly, without being locked into vendor-specific solutions. We support the establishment of NIST IAL2 as a federal standard and believe consumers should have the freedom to choose their preferred high-assurance, interoperable identity brand that puts them in control of their data.
   iii. Early, proactive communication is key to accelerating adoption and building trust with end users. CLEAR is happy to share best practices and work closely with CMS to support this initiative. CLEAR works closely with all partners on launch and go-to-market activities to optimize for adoption and end-user experience.

Best practices include an omnichannel approach to communications, highlighting value proposition and guiding users through the experience.

PR-9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

CMS could consider requiring Certified Electronic Health Record Technology **(**CEHRT) and CMS regulated payer Fast Healthcare Interoperability Resources (FHIR) services to demonstrate the ability for a patient to access services remotely. CMS could incentivize payer and provider adoption through a combination of MIPS requirements that such capabilities are enabled, and through Office of Civil Rights (OCR) guidance, that the use of such technologies avoids data breach reporting when otherwise required on account of an inaccurate patient match. Over time, the use of verified demographic data via CSPs that are used by Health IT systems should reduce the risks of relying on legacy probabilistic patient matching and manual data entry (i.e. call centers) and instead build more trustworthy 1:1 matching tied to the CSP that was involved in account creation to downstream uses, including data sharing networks.

CLEAR enables provider organizations to adopt identity verification through simple, secure, and low-lift implementations. Integration can occur at key access points like patient portal account creation, reducing IT overhead while improving security and user experience. The first time a patient verifies with a provider organization or network, CLEAR establishes a partner-specific "LinkID" that is unique to the patient's relationship with that given organization. When the patient returns for another experience (i.e. password reset, check-in, accessing health records) the "LinkID" is used to match 1:1 against an existing patient record, which has proven to reduce patient duplicates at the given organization.

CLEAR has invested in launching these experiences with core healthcare platforms including Epic. CLEAR is available in Epic's Showroom as "Under Construction" (integration underway) in Epic's new "Identity Verification for MyChart" Epic Toolbox category, enabling seamless and secure identity verification for patients. With CLEAR, health systems can offer a self-service experience for MyChart account creation and recovery — reducing administrative burden and fraud.

CLEAR is also a member of the Epic Connection Hub — supporting MyChart account workflows through a distinct integration path outside of Toolbox, and powering streamlined patient check-ins via Welcome.

CLEAR can help streamline proxy access for caregivers by automating the identity verification process that is typically completed in-person. Additionally, CLEAR is fully opt-in and requires partner organizations to maintain multiple options to support consumers who do not wish to use CLEAR. For individuals who are unable to verify or prefer to do so in-person, CLEAR is focused on ensuring that they can do so (i.e. airports) and is also exploring how to enable healthcare

providers (i.e. front desk staff) to facilitate a patient enrollment in CLEAR similar to how CLEAR Ambassador staff do at the airport.

a. **What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?**
    i. To accelerate the transition, providers may need:
        1. Implementation support (technical toolkits, funding, vendor-neutral guides)
        2. Assurances that verified IAL2/AAL2 credentials meet HIPAA compliance and cybersecurity requirements
        3. Encouragement from CMS through quality measures or participation requirements
    ii. Federal funding to implement digital identity verification solutions would accelerate adoption given budgetary constraints.
    iii. A federal requirement for NIST IAL2 (or similar) for certain use cases (i.e. account creation, account recovery) would accelerate the transition. Incentives would be important to ensure prioritization. Additionally, the Office of Civil Rights guidance could be used as a safe harbor for health systems that a) respond to patient Individual Access Services requests using IAL2 identity verification and b) write and store the CLEAR "LinkID" to in their EHR instance to improve patient matching when additional queries come through the same trusted health information exchange for the same patient that CMS participates in.
b. **How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?**
    i. Balancing privacy and convenience means ensuring credentials empower the patient while minimizing unnecessary data collection or surveillance. CMS should continue to center patient agency and choice with their preferred identity framework, similar to how consumers have choice in selecting their credit card.
    ii. Since day one, CLEAR's mission has been to make experiences safer and easier — physically and digitally. We started in the most highly-regulated environment, where nothing can come at the expense of security, and have expanded so that businesses can integrate our identity platform — without compromising the user experience. With over 31 million verified users on our platform, we have proven that security and convenience never have to come at the expense of the other.
    iii. CLEAR is committed to privacy, and is public with our Privacy Commitments. CLEAR never sells user data and meets the highest privacy and cybersecurity standards, including IAL2, AAL2, SOC 2 Type 2, PAD-2, and oversight from DHS and TSA.

PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

a. **What are the challenges and benefits for providers?**

i. **Operational Efficiency:** Digital identity credentials provide operational cost savings by reducing administrative burden and call center volume. These efficiencies drive immediate value for providers while supporting seamless digital front door strategies. For example, at Community Health Network, CLEAR drove a 1.5x higher verification success rate (93% vs. 62%) and reduced account-related support calls by more than 54%.

ii. **Reduced Fraud:** Verified digital identity strengthens identity assurance to prevent fraud and security breaches (i.e. a bad actor socially engineering access to a portal account via a call center).

iii. **Improved Matching & Fewer Duplicates:** Demographic information (i.e. name, DOB, address) are verified and corroborated with 3rd party sources, leading to a high provider match rate and reducing the likelihood of mismatches and duplicate records.

iv. **Implementation:** Providers would need to invest in upfront implementation and workflow changes including EHRs and Identity & Access Management systems, as examples. However, the benefits include streamlined account creation and recovery, reduced fraud risk, and stronger demographic-based patient / user matching thanks to verified demographic information (as opposed to call centers creating and looking up patients).

b. How would requiring their use improve access to health information?

i. Requiring digital identity credentials could unlock patient data across systems, particularly when paired with consumer-directed exchange standards, putting patients in full control of their information. Patients could access, share, and manage their information with greater ease and security, helping reduce healthcare's reliance on clipboards and self-attested data.

c. What are the potential downsides?

i. Health systems could incur an upfront investment to implement and drive adoption with a reusable federated identity platform. CMS could create incentives to implement this technology in order to help build the network. At scale, federated identity platforms drive network effects that would lower costs for the entire healthcare ecosystem.

d. What impact would mandatory credentials have on a nationwide provider directory?

i. Credentials can increase trust in provider directories by ensuring identity verification for providers themselves and their digital endpoints. If integrated with FHIR endpoint listings, identity credentials could help validate and maintain accurate, up-to-date directories.

ii. We would encourage IAL2 identity verification as a requirement to access certain nationwide provider directory features. The IAL2 standard is already being adopted for other provider use cases (i.e. The Joint Commission, Electronic Prescribing of Controlled Substances). If CMS required IAL2 for providers seeking to access Direct Primary Care or verify their NDH entry, it would allow for reuse of the existing IAL2-verified credential for other use cases, including at the provider's local health system (i.e. account recovery, EPCS). This would drive a

better provider experience, improve operational efficiency, and reduce fraud across the healthcare ecosystem.

iii. We would also encourage CMS to allow providers seeking to verify or update their nationwide provider directory listing via their preferred consumer platform (examples could include Epic, Surescripts or Symplr).

iv. CLEAR's work with Surescripts is a strong example of how the private sector has invested in solutions. Surescripts enforced IAL2 across their network—applying digital identity to protect directory integrity and secure healthcare communications. They have realized significant impact including:

1. 2x Higher Verification Success Rate: 80% with CLEAR vs. 41% with the legacy solution, a major improvement given IAL2's rigorous requirements.
2. 40% Faster Process: CLEAR replaced a lengthy, multistep process with a quick, one-time setup and instant verification with a selfie, unlocking faster activation and revenue potential.
3. 50% Boost in Pull-through: More providers are completing onboarding and getting to work faster

e. **How could digital identity implementation improve provider data flow?**

i. **Improved Matching & Fewer Duplicates:** Demographic information (i.e. name, DOB, address) are verified and corroborated with 3rd party sources, leading to a high provider match rate (CLEAR has seen >95% match rates with health system partners) and reducing the likelihood of mismatches and duplicate records.

ii. **Operational Efficiency:** Digital identity credentials provide operational cost savings by reducing administrative burden and call center volume. These efficiencies drive immediate value for providers while supporting seamless digital front door strategies. For example, at Community Health Network, CLEAR drove a 1.5x higher verification success rate (93% vs. 62%) and reduced account-related support calls by more than 54%. For a use case like patient check-in, front desk staff would spend less time looking up patients and scanning driver's licenses, and more time focused on higher-value tasks.

iii. **Reduced Fraud:** Verified digital identity strengthens identity assurance to prevent fraud and security breaches (i.e. a bad actor socially engineering access to a portal account via a call center).

PA-3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

CMS can accelerate the adoption of high-assurance digital identity credentials by aligning standards, incentives, and implementation support:

- Promote Standards-Based Credentials (e.g., NIST IAL2)
  - Incentivize payers to accept NIST IAL2-certified digital identities, ensuring trust, fraud prevention, and reusability. For example, CMS could tie acceptance of IAL2 credentials to programs like Blue Button 2.0, interoperability mandates, or administrative simplification initiatives.
- Require IAL2 for Sensitive Data Exchange

    - To protect sensitive health information and build ecosystem trust, CMS should limit payer-to-payer and patient-provider data sharing to identities verified at IAL2 or higher. This ensures that only high-assurance, standards-based credentials are used in consumer-mediated exchange.
- Support Credential Portability and Consumer Choice
    - Encourage identity portability across payers and services, reducing login fatigue and improving continuity of care. CMS should endorse federated identity frameworks that enable patients to choose and reuse verified credentials across the ecosystem.
- Fund Implementation of Early Adopter Programs
    - Provide targeted grants or technical assistance to help payers integrate certified third-party digital identity solutions. Use funding to drive adoption, evaluate performance, and share best practices.

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?
A nationwide provider directory would deliver substantial value to payers, particularly in security, data integrity, fraud prevention, and operational efficiency. A recent article in *Health IT News* points to the magnitude of the problem: The Council for Affordable Quality Healthcare (CAQH) "found that payers spend more than $2 billion and providers spend almost $2.8 billion on annual maintenance. Despite that colossal financial commitment, a CMS review of Medicare Advantage Organizations' provider directories found that slightly more than fifty percent of the provider directory locations had at least one inaccuracy."

An IAL2/AAL2 verified digital identity could serve as the foundation for a nationwide provider directory. Providers could verify their identity at IAL2 and, with their consent to share verified demographics, enable them to "discover" their credentials (i.e. medical licensure, education history, practice addresses). Potential benefits include:

- **Strengthen Trust and Prevent Provider Fraud:** Digital identity credentials tied to NIST IAL2 standards ensure that providers accessing or updating directory information are who they claim to be. Today, provider fraud and impersonation remain significant risks for payers, especially during credentialing and claims management. For example, a digitally verified identity credential (e.g., using biometrics and document proofing) tied to a provider's NPI ensures that only authorized individuals can access or modify sensitive information, mitigating risks of fraudulent data submission or access.
- **Security Impact:** Without verified digital identity, FHIR endpoint directories are vulnerable to spoofing, unauthorized access, or inaccurate routing.
- **Reduce Administrative Burden and Improve Accuracy:** Payers often face high costs and redundancy in validating provider identity across siloed systems. A national directory that couples digital identity with FHIR endpoints provides a single source of truth, reducing manual verification efforts, eliminating duplicate records, and speeding up onboarding and network maintenance.

- **Support Interoperable, Secure Data Exchange:** Enables payers to confidently exchange data with the correct entity. This ensures that clinical data, prior authorizations, or value-based performance measures are securely routed to the right provider, reducing leakage, rework, and risk.
- **Enable Deep, Actionable Identity Mapping:** A digital identity infrastructure enables identity with depth, linking real-world provider identities to critical data points that matter to payers, including: credentials and specialties, licensing and sanction history, network participation, etc. This allows payers to streamline provider onboarding, simplify data reconciliation, and improve coordination with clinical and administrative systems.

TD-3. Regarding digital identity implementation:

a. **What are the challenges and benefits?**
   i. Health systems could incur an upfront investment to implement and drive adoption with a reusable federated identity platform. CMS could create incentives to implement this technology in order to help build the network. At scale, federated identity platforms drive network effects that would lower costs for the entire healthcare ecosystem.

b. **How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?**
   i. CLEAR was built to protect personal information and keep users in control (learn more here). We started in a highly-regulated environment and maintain rigorous certifications on the commercial side of our business, including: IAL2, AAL2, SOC 2 Type 2, and PAD-2. Users explicitly consent to share their information with the end partner (e.g. CMS), making the data exchange transparent to both partners and end users.
   ii. Requiring digital identity credentials could establish a nationwide baseline of trust, helping Medicare beneficiaries navigate data access, care coordination, and benefits management more confidently and securely.
   iii. Cybersecurity improvements are significant: identity assurance mitigates credential theft, phishing, and impersonation. CLEAR's 97% workforce adoption at Community Health Network demonstrates that identity adoption is achievable—even in complex, high-volume settings.
   iv. Balancing privacy and convenience means ensuring credentials empower the patient while minimizing unnecessary data collection. CMS should continue to center patient agency and choice with their preferred identity framework, similar to how consumers have choice in selecting their credit card.

c. **What impact would mandatory use of the OpenID Connect identity protocol have?**
   i. **Establishing an Industry Standard:** We recognize that government mandates on specific technical standards runs the risk of picking the wrong standard with less agile means for correcting towards a better industry consensus approach. Today, OpenID Connect represents industry consensus and CLEAR fully supports it, and has deployed it across several partners. Mandating the functionality that OpenID Connect delivers as an interoperability standard for

identity would align healthcare with consumer tech standards and enable seamless federated access, while preserving the agility in the market for migration towards next generation approaches.

ii. **Interoperability:** OpenID Connect would advance Interoperability, as it plugs in with 3rd party platforms that support OAuth / OIDC standards.

iii. **Improved Security:** Access and identity tokens have tamper-proof expiry times, meaning if data were breached, there would be a limited window of usability by a bad actor. However, it's important to note that an expired token means that consumer health platforms may need to ask patients to reverify their identity a subsequent time to establish a new identity token. There are workarounds to this potential issue (i.e. if the partner application saves a "refresh token" at time of verification, they may continue to use it to request a refreshed identity token).

iv. **Standard Set of Demographic Attributes:** Identity tokens contain a standard set of claims (i.e. attributes about the user). The specific set of claims depends on the use case, but in all cases this will be a more limited set of information than everything collected during an identity verification transaction.